

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think>

Finding Reasoning Paths :

- 1. In attention , impulsivity , hyperactivity , and short attention span Attention - Deficit/Hyperactivity Disorder (ADHD) Behavior therapy or stimulant medications .
- 2. Frequent tantrums , aggression , and disruptiveness Behavioral abnormality Behavioral therapy , parental training , or pharmacological intervention .
- 3. Memory impairment , inattention , and distractibility Neuropsychological disorder (e.g . , ADHD , executive functioning deficit , or learning disability) Behavioral therapy or cognitive training .
- 4. Frequent ear infections Potential impact on hearing or cognitive development Rule out hearing impairment or other neurological conditions .
- 5. Family history of major depressive disorder and Graves ' disease Genetic predisposition to behavioral or psychiatric conditions Assess for underlying psychiatric or neurological disorders .

Reasoning Process :

- 1. Symptoms Analysis :
 - The child exhibits inattention , impulsivity , hyperactivity , and short attention span , which are hallmark symptoms of Attention - Deficit/Hyperactivity Disorder (ADHD) . ADHD is a common neurodevelopmental disorder in children and often presents with symptoms of inattention , hyperactivity , and impulsivity .
 - Additional symptoms such as frequent tantrums , aggression , and disruptiveness suggest behavioral challenges that may also align with ADHD or a conduct disorder .
 - Memory impairment , distractibility , and poor focus further support a diagnosis of ADHD or a related neuropsychological disorder .
- 2. Behavioral and Cognitive Evaluation :
 - The child’s symptoms (inattention , hyperactivity , impulsivity , and memory issues) point to a need for behavioral interventions . ADHD is typically managed with a combination of behavioral therapy (e.g . , parent training , social skills training) and pharmacological treatment (e.g . , stimulants like methylphenidate or non-stimulants like atomoxetine) .
 - Rule out other causes of inattention and hyperactivity , such as learning disabilities , auditory processing disorders , or neurological conditions . However , the absence of neurological findings on examination makes these less likely .
- 3. Family History Consideration :
 - The family history of major depressive disorder raises the possibility of a genetic predisposition to behavioral or psychiatric conditions . However , the child’s symptoms are more consistent with ADHD rather than mood or anxiety disorders .
 - The father’s Graves’ disease is unlikely to directly contribute to the child’s symptoms .
- 4. Frequent Ear Infections :
 - The history of recurrent ear infections at 2 years of age raises the possibility of residual hearing loss or auditory processing issues . However , the absence of hearing abnormalities on examination makes this less likely as the primary cause of the symptoms .
- 5. Management :
 - The most appropriate next step is to initiate behavioral therapy , focusing on addressing the child’s impulsivity , aggression , and disruptiveness . This may include parent training to improve behavioral management , social skills training , and cognitive-behavioral interventions .
 - If behavioral therapy alone is insufficient , pharmacological treatment (e.g . , stimulants or non-stimulants) may be considered . However , pharmacological intervention is typically reserved for cases with significant impairment after initial behavioral efforts .

Conclusion :

The most appropriate next step in treatment for this 5-year-old boy is behavioral therapy , focusing on addressing impulsivity , aggression , and disruptiveness . This approach aligns with the child’s symptoms and is consistent with